



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : منى عبدالنواب عيد مسعود
المحافظة : الجيزة
الحى / المدينة : الهرم
رقم البطاقة : 28002032301345 رقم التليفون : 01018570057
العمر (تاريخ الميلاد) : 03-02-1980 المهنة : ربه منزل
العنوان : 0
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور
قسم العلاج :
الأورام

[14163]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: منى عبدالنواب عيد مسعود National ID: 28002032301345	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

منى
عبدالتواب عيد مسعود
١٩ ش ابراهيم قنديل
الهرم - الجيزة

١٩٨٠/٠٢/٠٣ ٢٨٠٠٢٠٣٢٣٠١٣٤٥
KV8375969

كادسيلة

٢٠٢٤/١١ ٢٨٠٠٢٠٣٢٣٠١٣٤٥
ربة منزل

أنثى مسلمة متزوجة
سيد محمد احمد ابراهيم
البطاقة سارية حتى ٢٠٣١/١١/٠٥

١

03/10/2026

MDT FORM

Name:	Monna AbdelTawab Eid Masaoud	MRN:	5346
Age:	46 Y	ID:	28002032301345
Menopausal Sta	pre	Phone:	01141659384
NRS SCORE:		Marital Status:	married
BMI:		Family History:	negative
CO Morbidity:	asthmatic	Reason Of MDT:	post operative

Clinical Data:

BX: IDC 2 WITH MINIMAL DCIS ITERMEDIATE GRADE
ER: 7/8 PR: 6/8 HER2: 3+ KI67:

R

L

Sonomammography:

Mets Workup:

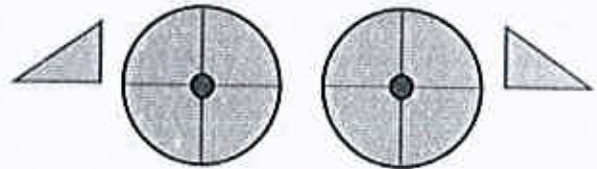
History:

Recommendations:

PT UNDERWENT LT MRM POST NACT WITH FREE SURGICAL MARGINS AND 3/12 L.N WITH PARTIAL PATHOLOGIC RESPONSE.
STAGE: T0, N1A

MDT Outcome & Plan

for adj Kadcyla 12 cycle PORT
ADJ femara/zoladex
zometa every 6 ms



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ ناني حسن		د/ ابو بكر حجازي			

Name: منى عبدالنواب عيد مسعود			
MR. : 5346			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 45-year-old with a history of pre-mastectomy radiotherapy for breast cancer, CT2 N2 disease, ER 7/8, PR 6/8, HER2 3+, REC TCHP, and an ejection fraction of 68% as of 27/01/2026. The patient underwent radiotherapy and modified radical mastectomy (MRM) with a planned adjuvant chemotherapy regimen of PHESGO, with an estimated 2/12 cycles completed by 02/12/2026. The multidisciplinary team (MDT) discussion on 21/02/2026 recommended adjuvant therapy with Kadcylla (12 cycles), a port for infusion, Zoladex, Femara, and Zometa for six months. On 18/03/2026 the patient was noted to have received neoadjuvant chemotherapy (NACT) followed by radiotherapy and MRM, achieving a partial response to Kadcylla (12 cycles) and PORT, with ongoing Femara, Zoladex, and Zometa for six months. On 18/04/2026 the patient was on cycle 2 of Kadcylla (260 mg) and continuing Femara and Zoladex, with a due date of 10/06/2026; the patient's ejection fraction was 66% as of 07/04/2026 and the patient was noted to be on the port. On 24/06/2026 the patient was on cycle 4 of Kadcylla (260 mg) and Femara; a dental examination was performed for Zometa, with the next Zoladex dose due on 01/08/2026. On 03/08/2026 the patient was on cycle 6 of Kadcylla (260 mg) and Femara, with supplemental treatment including paracetamol and Zometa; laboratory results were within normal limits. The patient has completed six cycles of Kadcylla to date.			
تحريراً في ,			
		التاريخ : 20-09-2026	الوقت :
		dina makram	الطبيب المعالج :
يعتمد ,			

Patient Name	: الأستاذ / منى عبدالقواب عيد مسعود	Request No.	: 13003238
Patient No.	: 28002032301345	Request Date	: 19/08/2025
Path No.	: C-262/25	Verification Date	: 24/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data
Right thyroid nodule, TIRADS4.

Nature of specimen
FNAC.

Gross specimen
Four alcohol fixed smear slides stained with H&E received for cytological examination.

Microscopic examination
-Adequacy : Adequate smears.
-Follicular cells :Present.
-Colloid :Present , Thick
-Nuclear enlargement :Absent
-Arrangement : Microfollicular and macrofollicular formation.
-Nuclear grooving : Absent
-Nuclear pseudo-inclusion :Absent
-Nuclear crowding /overlapping : Absent.
Additional pathological findings :
RBCS
*Scattered inflammatory cells .

Diagnosis
Right thyroid nodule, FNAC:
-Beingn Aspiration.
-Besthda category group II
Comment: The BETHESDA System for reporting thyroid cytopathology:

Verified by :



Patient Name : الأستاذة / منى عبدالنواب عيد مسعود
Patient No. : 28002032301345
Path No. : C-262/25
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13003238
Request Date : 19/08/2025
Verification Date : 24/08/2025
Financial Category : جوسفات روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

diagnostic categories:

- I. Indeterminate or Unsatisfactory.
- II. Benign. Consistent with a benign follicular nodule (includes adenomatoid nodule, colloid nodule, etc.).
Consistent with chronic lymphocytic (Hashimoto) thyroiditis in the proper clinical context.
Consistent with granulomatous (subacute) thyroiditis.
- III. Atypia of undetermined significance or follicular lesion of determined significance.
- IV. Follicular neoplasm or suspicious for follicular neoplasm.
- V. Suspicious for malignancy.
- VI. Malignant.

Signature
Prof. Dr.Gamal Abdel Atty
Dr.Walaa Ibrahim Ghoneim

Verified by :

Printing Date : 25/08/2025 11:44:59

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / مكي عبدالوهاب عيد مسعود	Request No. : 13004336
Patient No. : 28002032301345	Request Date : 05/07/2025
Path No. : SR-920/25	Verification Date : 07/07/2025
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 45 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Right breast mass

Nature of specimen

Block & slide revision

Gross examination

One block labelled by (202069) & 3 slides were received

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
- * DCIS: Minimal , cribriform pattern, intermediate grade
- * Angioinvasion: Absent
- * Perineural invasion: Absent
- * Additional pathological findings : Positive microcalcification
- * IPT (Positive control showed appropriate reactivity) :
- ER: Moderate nuclear reaction in 90 % tumor cells (Allred score 7/8)
- * PR: Strong nuclear reaction in 30 % tumor cells (Allred score 6/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)

Diagnosis

Right breast mass, block & slide revision:

Invasive duct carcinoma, Grade 2 with minimal Ductal carcinoma insitu , intermediate grade

IPT:

- * ER: Positive (score 7/8).
- * PR: Positive(score 6/8).

Verified by :



Patient Name : الأستاذ / منى عبدالقواب عيد مسعود
Patient No. : 28002032301345
Path No. : SR-920/25
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13004336
Request Date : 05/07/2025
Verification Date : 07/07/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

* HER2: Positive (score 3+).

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba Hassan Adly

Verified by :

Printing Date : 25/08/2025 11:44:59

Page 2 of 2

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ا.د. أحمد كمال الدري
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

NAME: MONA ABD EL TAWAB EID MASOUD
DEAR SIR

ID: 1344570

DATE: 24/07/2025

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- 45-year-old lady presented with right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings

- No evidence of bone secondaries could be seen.
- Marked degenerative changes are noticed at the spine and big joints.

IMPRESSION

- The skeleton is free from metastases.

Much obliged,
Dr. Ahmed NASR, MD
Dr. Alaa Nagi, MD
Nuclear Medicine

PATIENT NAME: MONA ABD EL TAWAB AID MASOUD

GENDER: Female

DATE : 18-Jun-2025 02:16 PM

PATIENT ID: 20225020287

AGE: 45 y

ORDER: Z207222,Z207223

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND**INDICATION:**

- Right breast palpable lump

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR b).

Right breast

- Right breast outer (9:00) middle third irregular spiculated high density mass with internal and external UOQ pleomorphic microcalcification in is seen segmental distributed toward nipple separated by 4 mm whole process measures 14x5.9 cm

Complementary Ultrasound revealed:

- Right 9 o'clock irregular spiculated vascular hypoechoic mass measuring 2.8 x3.2cm , 5.4 cm from nipple , 8 mm from the overlying skin, 5.4 mm from underlying muscle . Multiple irregular hypoechoic nodules are seen scattered from 8:00 to 11:00 nearest to nipple is 3.4 cm apart measures 3.4x4.9 mm
- Normal skin thickening and mild retracted nipple
- Right multiple pathological axillary level (I) lymphnodes (>3) some shows lost hilum others diffuse cortical thickening reaching 4 mm largest
- Single right axillary level (II) subpectoral showing eccentric hilum
- No pathological axillary level (III), internal mammary or supraclavicular lymphnodes.

Left breast

- No spiculated mass or grouped microcalcifications
- Left UOQ internal mammary lymphnodes
- This was confirmed by complementary US which revealed no left breast solid or cystic mass lesions.
- Dilated left retroareolar ducts reaching 5 mm with no internal contents
- Normal left skin thickness and contour.
- Normal left nipple areola complexe.
- No pathological axillary level (I) , (II) , (III) or internal mammary lymphnodes
- Prominent supraclavicular lymphnodes

OPINION:

- Right breast 8-11 o'clock malignant looking process with ipsilateral axillary level (I) and (II) pathological lymphnodes for Histopathological correlation (BIRADS 5)
- Right Benign breast findings (BIRADS2).

PATIENT: MONA ABD EL TAWAB AID MASOUD, 20225020287, Female, 45 y

Report version 2, 25-Jun-2025 10:11 AM

Page 1 of 2



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16302

بوهيا
مركز الأشعة
مركز الأشعة
مركز الأشعة

Patient Name: Mona Abd Al Tawwab Eid	Date : 20/07/2025
Exam : US Neck	Age : 45 Year(s)
Medical ID: 1898/2025	Pat. ID: 110624

Neck Ultrasonography

- The right and left lobes and isthmus of thyroid gland are of normal size, homogenous echopattern, with regular outline.
- Right thyroid lobe solid hypoechoic nodule with internal cystic area measuring 12x10mm (TIRADS 4) for follow up
- Normal vascularity at CCDI.
- The gland moves freely with deglutition.
- The trachea is centralized.
- Both carotid sheaths are free.
- Bilateral multiple enlarged cervical lymph nodes all with oval shape and echogenic hilum, the largest is at left side level III measuring 20x6mm likely non specific
- Prominent left supraclavicular LN measuring 10x6mm for follow up

Conclusions

Right thyroid nodule (TIRADS 4) for follow up

Signature Dr. Nermien Ramadan

20/07/2025 4:04 PM

PATIENT NAME: MONA ABD EL TAWAB AID MASOUD

GENDER: Female

DATE : 21-Jun-2025 03:38 PM

PATIENT ID: 20225020287

AGE: 45 y

ORDER: Z207426,Z207427

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC HEPATIC STUDY**

FINDINGS:

1- CHEST:

- Right breast LOQ enhancing mass lesion measuring about 27 mm associated with clumped segmental non mass enhancement reaching the nipple.
- Multiple pathologically enlarged right level I & II axillary lymph nodes; the largest is measuring about 26 x 10 mm.
- The left breast is partially outside the field.
- Prominent left supraclavicular lymph nodes, the largest measuring about 6 x 10 mm.
- Right lower lobe posterior basal segment small nodule measuring about 7 mm.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- Minimal pericardial effusion.
- Right thyroid lobe hypodense nodule for ultrasound correlation.

2- ABDOMEN AND PELVIS:

- The liver is enlarged with homogenous CT texture. Segment IV few calcific foci likely post granulomatous.
- No hepatic focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I. V.C and the pelvic organs.

PATIENT: MONA ABD EL TAWAB AID MASOUD, 20225020287, Female, 45 y

Report version 2, 21-Jun-2025 06:54 PM

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16602

مركز بهرية الأشعة
وخاصة أمراض الصدر والجهاز الهضمي
والمسالك البولية
والجهاز التناسلي
والجهاز العضلي الهيكلي
والجهاز العصبي
والجهاز المناعي

ECHO REPORT

NAME: منى عبد الثواب عبد سعود DATE: 2 / 12 / 2025
ID NUMBER: / SEX: F AGE: 45

EF	68.1	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	5.1	3.5-5.8cm	AO.R	2.5	3-3.7cm
LVESD		2-4cm	LA	3.4	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function DD grade I

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

Jalil

Patient Name : الأستاذ / منى عبدالنواب عيد مسعود
Patient No. : 28002032301345
Path No. : 240/26
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13004717
Request Date : 27/01/2026
Verification Date : 02/02/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer post NACT .

Nature of the specimen: Right breast & right axilla (MRM) (Post NACT).

Gross Examination:

- Procedure: Right modified radical mastectomy (MRM) measured 28x20x5 cm with covering skin ellipse measured 22x12cm.
- Tumor extent : A greyish white tumour bed site
- Size: 5 x3 x2 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 3 cm away from deep margin
- Axilla : Fibrofatty tissue piece measured 10x8x3 cm entangling 12 nodules up to 1.5x1 cm

Block designation :

1:5 TBS 6.Nipple 7.Deep margin 8.:11 ALNs

Grossing done by: Dr.Heba El-Shafeey .

- Microscopic examination:
- Histological type: No residual viable tumor cells, Cellularity 0%).
- Ductal carcinoma in situ: Absent
- Skin involvement: Free nipple and areola
- Lymph node involvement: Positive metastatic deposits in three lymph nodes out of twelve lymph nodes (3/12)
- Resection Margins: Free deep margin.
- Histologic response to therapy: Pathologic partial response

Diagnosis:

Right modified radical mastectomy, MRM (post NACT) :

No residual viable tumor cells , viability 0 %

- Deep margin:Free of tumor cells
- Nipple and areola :Free of tumor cells

Verified by :

Dr.Heba Hasan

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Patient Name : الأستاذ / مني عبدالنواب عيد مسعود
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Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Aillary lymph nodes :Positive for metastatic deposits in three out of twelve lymph nodes (3/12).
- Pathologic partial response
- RCB : 1.675 , RCB class II, viability 0 %
- Stage : ypT0 , ypN1a

Signature
Prof. Dr. Maha Guimei
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

Printing Date : 03/02/2026 14:39:16

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